

# DECODED EXPLAINED

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## One in Ten Indians Faces Risk of Cancer Before 75: Here Is What WHO Data Reveals

# One in ten Indians faces risk of cancer before 75: Here is what WHO data reveals

**Anuradha Mascarenhas**  
*New Delhi, July 9*

CANCER IS steadily becoming one of India's defining health challenges. According to the latest GLOBOCAN estimates, nearly one in ten Indians is at risk of developing cancer before the age of 75, while about seven in every hundred face the risk of dying from the disease before reaching that age.

India recorded 1.41 million new cancer cases and 916,827 deaths in 2022, while more than 3.25 million people were living with a cancer diagnosis made within the previous five years.

The burden is already increasing. According to estimates presented in the WHO Global Status Report on Cancer 2026, India had approximately 1.6 million new cancer cases in 2024, with around 900,000 deaths. Experts now project that annual new cases could

climb to 2.8 million by 2050, driven by population growth, ageing and changing lifestyles.

India and China together account for more than half of the global cancer burden, making Asia the epicentre of the disease. Without stronger prevention and early detection, experts warn the region will shoulder an even larger share of the global burden in the coming decades.

India's growing cancer burden is due to multiple forces acting together. One is the increase of life expectancy as it rises so does the number of people vulnerable to cancer.

Equally important are lifestyle changes accompanying rapid urbanisation. Rising obesity, unhealthy diets, reduced physical activity, excessive alcohol consumption and prolonged exposure to air pollution are contributing to increasing rates of several

cancers. Smoking continues to drive lung cancer, while smokeless tobacco products — including gutkha, khaini and betel quid — remain a major reason India has one of the world's highest burdens of oral cancer. Improved diagnostic facilities have also resulted in more cancers being detected than in previous decades; experts stress that this does not fully explain the rising incidence.

Dr Isabelle Soerjomataram, Deputy Head of the Cancer Surveillance Unit at the International Agency for Research on Cancer (IARC) said, prevention has become important in India because lung, oral cavity, cervical, breast and colorectal cancers account for a substantial proportion of the country's disease burden. She points to stark differences in access to

healthcare between metropolitan centres and nearby rural districts, where delayed diagnosis is common. India's cancer pattern differs significantly from that seen in many Western countries.

Breast cancer emerged as the country's most common cancer, accounting for 192,020 new cases in 2022, followed by lip and oral cavity cancer (143,759 cases), cervical cancer (127,526), lung cancer (81,748) and oesophageal cancer (70,637).

Among women, breast cancer accounts for more than one in four new diagnoses, while cervical cancer remains the second leading cancer despite being largely preventable through HPV vaccination and regular screening.

## One in Ten Indians Faces Risk of Cancer Before 75: Here Is What WHO Data Reveals

- Cancer is a disease in which some of the body's cells grow uncontrollably and spread to other parts of the body.
- Normally, human cells grow and multiply (through a process called cell division) to form new cells as the body needs them. When cells grow old or become damaged, they die, and new cells take their place.
- Sometimes this orderly process breaks down, and abnormal or damaged cells grow and multiply when they shouldn't. These cells may form tumors, which are lumps of tissue.
- Cancer is a genetic disease, that is, it is caused by changes to genes that control the way our cells function, especially how they grow and divide.
- Genetic changes that cause cancer can happen because:
  - of errors that occur as cells divide.
  - of damage to DNA caused by harmful substances in the environment, such as the chemicals in tobacco smoke and ultraviolet rays from the sun. (Our Cancer Causes and Prevention section has more information.)
  - they were inherited from our parents.

## One in Ten Indians Faces Risk of Cancer Before 75: Here Is What WHO Data Reveals

- **The Global Status Report on Cancer 2026** provides a comprehensive assessment of the global cancer burden, regional trends, healthcare preparedness, and policy responses. It also identifies priority actions needed to improve cancer prevention, diagnosis, treatment, survivorship, and palliative care.
- Published by: Jointly released by the World Health Organization (WHO) and the International Agency for Research on Cancer (IARC).
- Around 20.6 million new cancer cases and nearly 10 million deaths occur every year, making cancer the second leading cause of death globally after cardiovascular diseases.
- Asia accounts for more than half of global cancer cases and deaths.
- Lung cancer remains the leading cause of cancer deaths globally.
- Reasons for increased burden of Cancer
  - Population ageing, Lifestyle-related risk factors, Infection-related cancers, Environmental exposures, Urbanisation and changing lifestyles, slow progress in prevention: Although tobacco control, vaccination programmes, and public health policies have reduced certain cancer risks, progress has not kept pace with emerging challenges such as obesity, sedentary lifestyles, and environmental pollution.
  - Preventable risk factors remain widespread: The report estimates that nearly four in ten cancer cases are linked to preventable risk factors, highlighting the need for stronger preventive healthcare and health promotion measures.

## One in Ten Indians Faces Risk of Cancer Before 75: Here Is What WHO Data Reveals

- **Initiatives against Cancer**
  - **National Programme for Prevention and Control of Cancer, Diabetes, Cardiovascular Diseases and Stroke (NPCDCS)**
  - **National Cancer Institute (NCI) in Jhajjar, Haryana**
  - **Ayushman Bharat Yojana**
  - **The Health Minister's Cancer Patient Fund (HMCPUF):** The Health Minister's Cancer Patient Fund under Rashtriya Arogya Nidhi (RAN) provides financial aid up to ₹5 lakh for cancer treatment to patients below the poverty line.
  - **National Cancer Grid (NCG):** The National Cancer Grid (NCG) was established in 2012 to ensure high-quality, standardized cancer care across India. Eight years later, it has grown into the world's largest cancer network with 287 members
  - **In Sep 2024, India, in partnership with the US, Australia, and Japan, has launched the Quad Cancer Moonshot to eliminate cervical cancer across the Indo-Pacific region**



# Beyond Childcare, Four Other Barriers

## Beyond childcare, four other barriers



ROOPA KUDVA

**T**HERE ARE two things worth cheering about in the latest report of the National Statistics Office on India's million-plus cities. First, female labour force participation has climbed from 19.8 per cent in 2017-18 to 27.2 per cent in 2025, showing the sharpest gain of any group. The second is entrepreneurship — in 32 of the 46 million-plus cities, more than 20 per cent of unincorporated establishments are now run by women. Cities have become India's biggest centres of opportunity for women — and increasingly, it isn't only the big metros. In Surat, Vadodara and Pune, 40 per cent of unincorporated establishments are run by women.

Both numbers deserve celebration, but they come with a catch. Female LFPR, even after this gain, is still barely a third of the male rate of 75.9 per cent. Startlingly, despite the shift in aspirations, nearly 70 per cent of women outside the labour force still cite childcare and home commitments as the reason, proof that this is far from a solved problem.

For the rest, however, four gaps explain why the LFPR gap is not closing faster. Every system that touches a woman's working life — how she gets to work, whether she can occupy public space with the same ease as men, whether she rises once she's in an organisation, whether a bank or an investor will fund her — is designed around a male default.

**Mobility:** For men, the labour market begins at the workplace. For many women, it begins much earlier — on the street, at the bus stop, at the train station. ORF's Women on the Move study found that over half of women reported experiencing harassment on public transport, and a similar share perceive it as unsafe. Women also rarely have linear commutes — their journeys are "chained", dropping children to school, managing errands, on the way to or back from work — while our transport systems are still built around a simple home-to-office trip. If transport isn't reliable, safe or built for how women actually move through a city, labour supply shrinks before an employer ever sees the candidate.

**Public spaces:** The deeper problem is beyond the commute. Look at our public spaces — you rarely see women simply sitting in a park alone, lingering in a café, or

spending time on a street without a clear purpose. Women feel they must be doing something legitimate to justify their presence in public space. That's a very different problem from safety alone, and it's one policing can't fix. The answer is designing cities that assume women are full participants in public life — not those who need a reason to be there.

**Career progression:** Even women who clear the first two hurdles run into a third: They don't rise at the rate they enter. Only 5 per cent of NSE-500 companies are women-led. For years, the explanation has been maternity breaks and caregiving. That narrative is tired and incomplete — it explains some exits, but not why so many women who stay still don't progress into leadership.

A large part of the answer is unconscious bias. Successful women are often less liked than successful men, because traits like assertiveness and driving accountability conflict with longstanding expectations that women be accommodating and nurturing. A firm view from a man reads as conviction; from a woman, as sharp. Women are asked to deliver outcomes while continuously managing tone and perception — and that shapes self-belief and choices, sometimes unconsciously. That is why, as one report showed, men apply for roles when they meet 60 per cent of the qualifications, while women wait until they meet nearly all of them.

This is where the conversation usually runs into a familiar objection — that supporting women somehow means compromising merit. Far from it. Promoting women isn't anti-merit; it's how you actually give yourself the chance to uncover more of it. Organisations that effectively exclude half the workforce aren't protecting standards. They're narrowing them.

**Funding:** The fourth gap is capital. Women-led MSMEs in India face a credit gap of around 35 per cent, held back by inadequate collateral, lower property ownership and financial literacy. Further up the ladder: Less than 5 per cent of India's VC funding goes to start-ups with a woman founder. Part of the reason is pitch-room dynamics. Studies have shown that investors ask women founders more "prevention" questions — about risk, safety, what could go wrong — while men are asked "promotion" questions, about growth and scale.

Cities have opened a door for women that didn't exist a decade ago. But walking through it still means clearing a commute, a public square, a boardroom and a bank. Until all four are rebuilt for her, closing the gap with men will take far longer than it needs to.

*The writer headed Crisil and ONI*

## Beyond Childcare, Four Other Barriers

- **Labour Force Participation Rate**

- **Labour Force Participation Rate (LFPR):** the percentage of persons in the labour force (working or seeking or available for work) in the population.
- **Worker Population Ratio (WPR):** defined as the percentage of employed persons in the population.
- **Unemployment Rate (UR):** UR is the percentage of persons unemployed among the persons in the labour force.

## Beyond Childcare, Four Other Barriers

- **Positive Signals**

- Female labour force participation in India's million-plus cities rose from 19.8% (2017-18) to 27.2% (2025); in 32 of 46 million-plus cities, over 20% of unincorporated establishments are women-run

- **Still a Wide Gap**

- Female LFPR remains barely a third of the male rate (75.9%); nearly 70% of women outside the labour force still cite childcare and home commitments as the reason

- **Barrier : Mobility**

- Women's commutes are "chained" (dropping children, errands) rather than linear; over half report experiencing harassment on public transport, discouraging labour supply

- **Barrier : Public Spaces**

- Women feel they must justify their presence in public space with a clear purpose a problem distinct from safety alone, requiring cities designed around women as full participants

- **Barriers : Career Progression and Funding**

- Only 5% of NSE-500 companies are women-led, partly due to unconscious bias; women-led MSMEs face a credit gap of around 35%, and under 5% of India's VC funding goes to women-founded start-ups



# Why Is the Centre Revising the NFSA?

## Why is the Centre revising the NFSA?

Why has the Centre proposed changing AAY foodgrain entitlements? How will the proposed amendment affect households? Why are Tamil Nadu and Kerala opposing? Could the amendment widen regional disparities in foodgrain allocation?

### EXPLAINER

T. Ramakrishnan

The story so far:

On July 6, Tamil Nadu Chief Minister C. Joseph Vijay urged the Centre to retain the present provision of 35 kg of foodgrains per household per month under the Antyodaya Anna Yojana (AAY) meant for the poorest of the poor, and not to make it a per capita system. The next day, the CPDM Polit Bureau voiced its concern and demanded that the proposed amendment to the entitlement criteria be dropped. About 10 days ago, immediately after the Union government made public its plan to amend the National Food Security Act (NFSA), Kerala's Food Minister Anoop Jacob expressed reservations over the move.

What is the amendment mooted?

On June 24, the Union Food and Public Distribution (F&PD) Department, while publishing a draft amendment bill to the NFSA, said every person belonging to AAY households would be entitled to seven kg of foodgrains per month, subject to a maximum of 35 kg per household per month. At present, the entitlement is for the entire household with a ceiling of 35 kg per month. The proposed amendment covers the first proviso to sub-section (1) of Section 3 (Right to receive foodgrains at subsidised prices by persons belonging to eligible households) of the Act. The public can comment on the amendments till July 13 and send their views by email to [suneel.sachdeva@nic.in](mailto:suneel.sachdeva@nic.in) and [saurabhmar@ibggn.in](mailto:saurabhmar@ibggn.in).

Why is the change being proposed?

The existing household-based entitlement, though intended as a protective measure for the most vulnerable families, results in significant inequities depending upon the size of the household, according to the F&PD Department. Smaller households receive a higher per-capita entitlement, whereas larger households receive a lower per-capita entitlement, which may fall below the entitlement available to priority households.

The aim and purpose are to remove intra-category inequities, provide for more rational foodgrain allocation, and better align entitlements with nutritional requirements, a note prepared by the department said.

However, the proposed amendment does not seek to address the inclusion of ineligible persons as beneficiaries, a problem that persists at the State level.

What is the story behind the two southern States' opposition?

It is not for the first time that the two States are articulating their opposition to matters concerning the food policy, as their contemporary political history has an important ingredient – the politics of food. Kerala, which has long been the public distribution system (PDS) dating back to the now-abolished princely State of Travancore, introduced informal food distribution mechanisms to mitigate the food shortage of the poor and vulnerable, and was perhaps the first to launch a formal PDS in 1962, three years before the establishment of the Food Corporation of India (FCI). At least on two occasions – 1952 and 1967 – Tamil Nadu saw political upheavals, the incumbent regime getting reduced to a minority in 1952 and being shown the door (in 1967). The reason was that the governments of

### Grains of debate

The Centre's proposed amendment to the National Food Security Act has triggered concerns that foodgrain allocations under the Antyodaya Anna Yojana could fall for many households in the southern States



Table 1: AAY coverage, beneficiaries and foodgrain allocation

| State              | Ceiling of foodgrains per household (in lakh) | AAY coverage No. of AAY households (in lakh) | No. of beneficiaries (in lakh) | Allocation of foodgrains** per 2024-25 (in tonnes) |
|--------------------|-----------------------------------------------|----------------------------------------------|--------------------------------|----------------------------------------------------|
| Andhra Pradesh     | 9.09                                          | 8.48                                         | 22.48                          | 3,81,557                                           |
| Karnataka          | 12                                            | 18.1                                         | 42.12                          | 4,86,748                                           |
| Kerala             | 5.96                                          | 5.93                                         | 18.53                          | 2,50,236                                           |
| Tamil Nadu         | 18.65                                         | 18.57                                        | 62.23                          | 7,82,771                                           |
| Telangana          | 6.49                                          | 5.64                                         | 15.66                          | 2,28,074                                           |
| Puducherry (U.T.)  | 0.32                                          | 0.24                                         | 0.77                           | 0*                                                 |
| Share of the South | 52.51                                         | 49.36                                        | 162.37                         | 21,13,378                                          |
| All India          | 290                                           | 217.81                                       | 777.41                         | 99,33,962                                          |

Source: Foodgrain bulletin for May 2024, Department of Food and Public Distribution, Government of India

\*No Puducherry follows the concept of direct benefit transfer; no allocation has been made.

\*\* Foodgrains – rice, wheat and coarse grains

**Fine balance:** The aim and purpose of the amendment are to remove intra-category inequities, provide for more rational foodgrain allocation, and better align entitlements with nutritional requirements. M. Srinivas

Table 2: Off-take and distribution in the Southern States

How the South fared during the current financial year up to the month of May 2024 (tonnes)

| State              | Off-take  | Requirement | Off-take (Current) | Distribution |
|--------------------|-----------|-------------|--------------------|--------------|
| Andhra Pradesh     | 53,593    | 63,593      | 57,837             | 57,837       |
| Karnataka          | 76,799    | 57,316      | 35,003             |              |
| Kerala             | 41,706    | 39,812      | 20,458             |              |
| Tamil Nadu         | 1,30,462  | 1,30,135    | 1,21,709           |              |
| Telangana          | 39,679    | 59,515      | 38,893             |              |
| Share of the South | 3,52,200  | 4,10,975    | 2,73,900           |              |
| All India          | 18,55,509 | 17,24,189   | 13,75,521          |              |

the periods in question were not being swift in handling the rice shortage. Since 1967, successive Chief Ministers of the State have been cautious in taking decisions with regard to the subject of rice.

It was no wonder that the two States were vociferous in their stand in the run-up to the formulation of the NFSA. Though the Congress-led United Democratic Front (UDF) took charge in 2001 in Kerala, the government did not agree to the implementation of the 2003 law – an Act of Parliament – despite the Congress-led United Progressive Alliance regime at the Centre pushing the legislation. The main reservation it had was that the NFSA would lead to the removal of a "large number of poor families" from the list of beneficiaries and put an "enormous financial burden" on the State. However, the then Chief Minister, Oommen Chandy, at one point in time, committed himself to the enforcement of the law in his State, though it was left to his successor, Pinarayi Vijayan, to take the formal decision.

In Tamil Nadu, Jayalalithaa, as Chief Minister, virulently opposed the law because immediately after she came to power in May 2010, her government started the distribution of rice, free to all the ration cardholders, regardless of their economic status. She had criticised the government for stating that those who would be left out of the NFSA would not

The existing household-based entitlement, though intended as a protective measure for the most vulnerable families, results in significant inequities depending upon the size of the household

be eligible to receive free rice. Eventually, she extracted a major concession from the Union government that the existing allocations, as they stood in 2013, for all the States would be legally safeguarded.

Hence, it was no surprise that the two southern neighbours joined the rest of the country in November 2016 in implementing the law.

Why are Tamil Nadu and Kerala opposing the amendment to NFSA?

Explaining how Kerala would be affected by the latest move, Mr. Jacob has pointed out that States such as his, characterised by nuclear families, will be at a disadvantage as there will be a reduction in the quantity of free foodgrains for families with fewer than five members. He has recalled that even in 2013, when the law took effect, his State took the stand that the AAY cardholders deserved "special consideration" – a position that it continues. "As a consumption State, any cut in Kerala's allocation is a matter of concern," Mr. Jacob stated. Mr. Vijay, who referred in his letter to Mr. Modi about how the amendment

would cause a fall in the monthly allocation of foodgrains from 65,261 tonnes to 42,040 tonnes, stated that the number of AAY cardholders below the family size of five members is 15.75 lakh out of a total of 18.64 lakh, covering 55.51 lakh beneficiaries (total: 69.27 lakh). "Rice provided to the AAY cardholders is a staple ingredient of all three meals of the day and cannot be substituted with any other commodity from the open market, resulting in substantial out of pocket expenses," he said.

Besides, Anuraधा Thapar, a functionary of the Right to Food Campaign, said that the amendment would bring a "North-South divide" in foodgrain allocation, as families in northern States would get higher allocations as their average family size was bigger than that of the southern States.

What is the way forward?

Ideally, such a change should have been subjected to greater public scrutiny to arrive at a consensus. However, T. Sasikumar, president, Tamil Nadu Progressive Consumer Centre and veteran activist who served on the State government's panel on food, suggests that the Centre chalk out a middle path by making the allocation of 30 kg per family, irrespective of family members. This would help the Union government to cut down its subsidy bill.

### THE GIST

The Centre has proposed amending the NFSA to change AAY foodgrain entitlements from a household-based system to a per capita allocation of 7 kg per person, subject to a maximum of 35 kg per household, to address inequities in the existing system.

Tamil Nadu and Kerala have opposed the proposal, arguing that it will reduce foodgrain allocations for households with fewer than five members and increase the burden on poor families, particularly in States with predominantly nuclear families.

While the Centre says the amendment will make foodgrain distribution more equitable, critics warn it could widen regional disparities, and some have suggested a middle path of a fixed 30 kg allocation per household.

## Why Is the Centre Revising the NFSA?

- The Act covers up to 75% of the rural population and up to 50% of the urban population for receiving subsidized good grains under the Targeted Public Distribution System (TPDS), thus covering about two-thirds of the population.
- Under NFSA, the government is obliged to give subsidized food grains every month to the beneficiaries identified by each state government adhering to inclusion/exclusion criteria set by the central government.
- Two categories of beneficiary households:
  - Antyodaya Anna Yojana (AAY): AAY households receive 35 kg of food grains monthly.
  - Priority Households (PHH): Each PHH member gets five kg of food grains monthly ( Rice, wheat and coarse grains) [Till 2028]
- The free nutritious meal to pregnant women and lactating mothers and children up to 6 years of age under the Integrated Child Development Services (ICDS) scheme.
- The free nutritious meal to children in the 6-14 age group under the Mid-Day Meal Scheme.
- Maternity benefits of Rs. 6000 to Pregnant Women and Lactating Mothers.

# Why Is the Centre Revising the NFSA?

- **The Proposed Amendment**

- On June 24, the Centre proposed changing Antyodaya Anna Yojana (AAY) entitlements from a household-based 35 kg/month allocation to a per capita allocation of 7 kg/person, subject to a 35 kg per household ceiling

- **Rationale**

- The existing household-based entitlement creates intra-category inequities — smaller households get a higher per-capita share while larger households get less; the amendment aims for more rational, nutrition-aligned allocation

- **Why Tamil Nadu and Kerala Oppose**

- Both states, with more nuclear families of fewer than five members, would see reduced allocations under a per-capita cap; critics warn of a "North-South divide" as northern states have larger average family sizes

- **Historical Context**

- Kerala and Tamil Nadu have long resisted uniform food security formulations, both having pioneered independent public distribution systems (PDS) before the NFSA (Kerala's PDS traces to 1962, pre-dating the FCI)

- **The Middle Path**

- Experts suggest a fixed 30 kg per household allocation, irrespective of family size, as a compromise that could still help the Centre reduce subsidy costs while avoiding disproportionate harm to smaller households

# Why More Indian Couples Are Being Diagnosed With Secondary Infertility

## Why more Indian couples are being diagnosed with secondary infertility

Analyses of India's National Family Health Survey data show secondary infertility rose from roughly 19.5% in the early 1990s to about 28.6% by 2015-2016 — close to a doubling in a single generation; there are multiple factors that are contributing to this including age-related issues and changes in the body between the first and the second child

Richika Sahay Shukla

A few weeks ago, a couple sat in my consulting room looking almost apologetic. They already had a bright six-year-old waiting at home. "We feel silly taking your time, doctor," the mother said. "We have one child. Everyone keeps telling us to just be grateful." And then, quietly, she started to cry.

I have heard that phrase — "at least you have one" — more times than I can count. It is almost always meant kindly. It almost always lands like a door being shut. And it is one of the reasons the couples I see struggling to conceive a second time carry their grief so silently.

### Understanding the condition

The condition has a name: secondary infertility — the difficulty conceiving or carrying a pregnancy after you have already had a baby. For years it was brushed aside as a non-problem. The numbers say otherwise.

Analyses of India's National Family Health Survey data show secondary infertility rose from roughly 19.5% in the early 1990s to about 28.6% by 2015-2016 — close to a doubling in a single generation. It is now one of the most common reasons a "previously fertile" couple walks in, bewildered. "We didn't even try the first time," they say. "Why is it so hard now?"

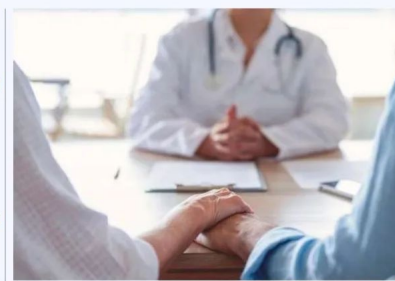
Part of the answer is the calendar. A generation ago, most couples finished their families in their twenties. Today a first baby often arrives at the age of 31 or 32 — after the degree, the

job, the EMI, the move to a rented flat in a city.

By the time life finally feels "ready" for a second child, the mother may be aged 36 or 38, and fertility, unfairly, does not wait for our life plans. Egg numbers and quality fall with every passing year; clinics in cities including Delhi/NCR, Bengaluru, Mumbai and others, have reported a sharp rise in women seeking help only after the age of 35.

But age is just one thread. In the years between the first and second child, bodies change. Weight creeps up. The thyroid quietly slips. As many as one in five Indian women now lives with Polycystic Ovarian Syndrome (PCOS), which tends to worsen with time. A Caesarean section may have left adhesions; an old, untreated infection may have scarred a fallopian tube. And — this matters — the problem is very often on the man's side. Sperm counts fall with weight, stress, smoking and even the polluted air so many of us breathe. Yet in my experience, Indian men will wait years longer than their wives before agreeing to a simple semen test, convinced that "we already have a child, so it can't be me."

Layered over all of this is something



**Growing concern:** Secondary infertility is the difficulty conceiving or carrying a pregnancy after you have already had a baby. GETTY IMAGES

newer: our phones. Social media has, in many ways, been a gift for fertility awareness. Couples now arrive far better informed than a decade ago; the taboo is finally cracking; brave women share IVF journeys that once would have been whispered about. But scroll through any feed and you also meet the other side — an unending stream of "Baby No. 2!" announcements, and picture-perfect siblings.

For a woman in her fourth month of trying, or recovering from a second miscarriage,

that scroll is a slow bruise. The comparison is relentless — and so is the family WhatsApp group where an aunt asks, yet again, when the "good news" is coming. I have also watched social media do harm the other way: glossy reels promising IVF "success in one cycle," or home remedies that persuade a couple to delay proper testing by a year they cannot afford to lose.

Telling the truth  
So what do I tell the couple crying in my room? First, that their pain is real and they are allowed to feel it.  
Wanting a sibling for the child you adore is not ingratitude; it is love asking

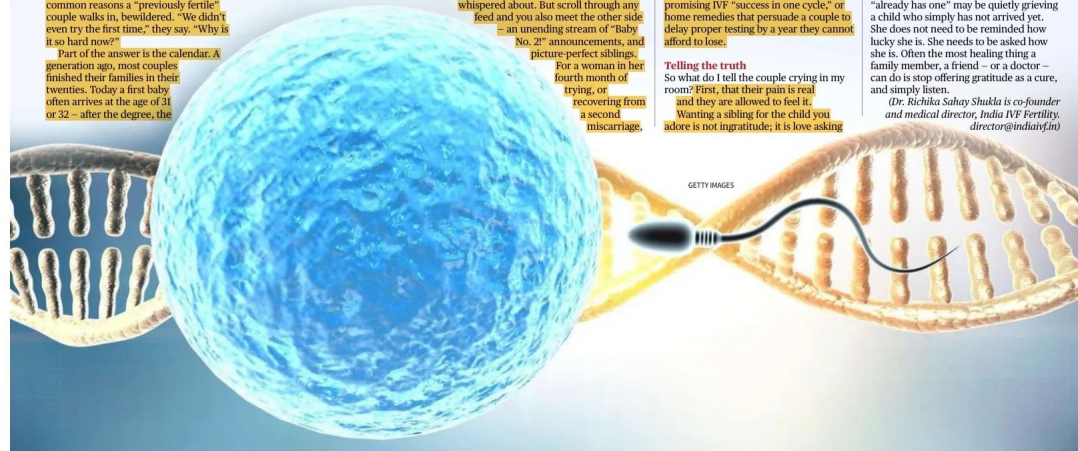
Egg numbers and quality fall with every passing year; clinics in cities including Delhi/National Capital Region, Bengaluru, Mumbai and others, have reported a sharp rise in women seeking help only after the age of 35

for a little more room. Second, that this is a medical condition, not a moral verdict — not a punishment, not near, not something they did wrong. Third, that timing matters. If a woman is under the age of 35 and has tried for a year, or over the age of 35 and has tried for six months, both partners — together — should be evaluated. Do not let the calendar run, out of politeness.

And I tell them the truth about hope, honestly. Modern fertility care helps a great many couples in exactly this situation, and IVF succeeds for many even in their late thirties and forties. But I will never promise a guaranteed outcome, and I would gently warn anyone against a clinic that does. What I can promise is a clear answer and a plan.

If there is one thing I wish more of us understood, it is this: the woman who "already has one" may be quietly grieving a child who simply has not arrived yet. She does not need to be reminded how lucky she is. She needs to be asked how she is. Often the most healing thing a family member, a friend — or a doctor — can do is stop offering gratitude as a cure, and simply listen.

(Dr. Richika Sahay Shukla is co-founder and medical director, India IVF Fertility, director@indiaivf.in)



# Why More Indian Couples Are Being Diagnosed With Secondary Infertility

- **Defining the Condition**

- Secondary infertility is difficulty conceiving or carrying a pregnancy after already having a baby; long dismissed as a non-problem, it now affects a growing share of couples

- **The Data Trend**

- Analyses of India's National Family Health Survey data show secondary infertility rose from roughly 19.5% in the early 1990s to about 28.6% by 2015-16 — close to doubling in a generation

- **Contributing Factors**

- Delayed first childbirth, age-related decline in egg quality after 35, weight and thyroid changes, Polyendocrine Metabolic Ovarian Syndrome (affecting roughly one in five Indian women), and male-side factors like falling sperm counts

- **Social and Behavioural Dimensions**

- Social media cuts both ways raising fertility awareness while also fuelling comparison and misinformation about "guaranteed" IVF success or delaying proper testing

# **Thank You**